

METRO NEUROSCIENCE & SURGICAL INSTITUTE

Department of Functional Neurosurgery & Movement Disorders

Patient Name: John Anderson	DOB: 03/22/1965 (Age 61)
Patient ID: pat-016	Gender: Male
Payer: BlueCross BlueShield	Primary Care: Dr. James Collins, MD
Requesting Physician: Dr. Marcus Thorne, MD	NPI: 1982736450
Procedure Requested: CPT 61863 / HCPCS L8679	Primary Diagnosis: G25.0 Essential Tremor

NEUROLOGICAL CLINICAL EVALUATION & PRIOR AUTH DOCUMENTATION

Clinical History & Diagnosis:

Patient is a 61-year-old male with a 14-month documented history of severe, progressive bilateral postural and kinetic hand tremor, formally diagnosed with **Essential tremor** (ICD-10 G25.0). The tremor causes severe functional disability, preventing him from writing, holding eating utensils, buttoning clothes, and performing basic activities of daily living (ADLs) independently.

Prior Pharmacological Therapy (Medical Treatment History):

Patient has completed maximal tolerated medical therapy for controlling tremor. He was prescribed first-line pharmacological agents including **Propranolol** (titrated up to 240 mg daily) and **Primidone** (titrated up to 750 mg daily) for over 6 months continuously. Despite compliance with maximal medical therapy, the tremor remained severe and functionally disabling without significant therapeutic improvement. Patient thus meets the criterion of **Failed maximal medical therapy for controlling tremor**.

Cognitive Assessment:

Formal neuropsychological testing recorded a **Mini-Mental State Examination score** of **28** out of 30 (MMSE score = 28, well exceeding the required threshold of ≥ 24). Patient exhibits intact cognitive function, orientation, memory, and executive reasoning, with no evidence of dementia or neurodegenerative cognitive decline.

Psychiatric & Depressive Symptoms Screening:

Neuropsychiatric evaluation confirms **severe depression: No evidence**. Hamilton Depression Rating Scale (HAM-D) score is 4 (normal/non-depressed). There is no past or current history of major depressive disorder, refractory obsessive-compulsive disorder, primary headache disorders, neuropathic pain syndromes, or active psychosis.

Duration of Clinical Documentation:

Neurological care, clinical notes, and physical examination findings for this tremor episode span **14 months** of continuous clinical notes duration (exceeding the required ≥ 6 months threshold).

Surgical Recommendation:

In summary, patient satisfies all clinical criteria for **Essential tremor thalamic stimulation**. Deep Brain Stimulation (DBS) thalamic lead implantation (CPT 61863) with neurostimulator pulse generator (HCPCS L8679) is strongly recommended for approval.

Attending Physician: Dr. Marcus Thorne, MD	Signature: M. Thorne, MD
Date: August 24, 2026	License #: MD-948102-NY